The introduction film

MedLYFE. First shown at AGELESS

FOR	The production company
FROM	MedLYFE Wellness and Longevity Centre, with Consult for Africa
DATE	Friday 9 October 2026

In short

- One film, 90 seconds, built to sell. It introduces MedLYFE and asks for a booking, and it has to keep doing both for months.
- First outing is 6:30pm on Wednesday 21 October at Capital Club, Lagos, as the evening opens.
- After that it is the standing introduction: the website hero, the first thing on every discovery call, reception, the sales deck, the WhatsApp forward.
- So nothing in it is about the event. No date, no venue, no guests, no AGELESS. It must not expire on the 22nd.
- It carries the discovery deck's spine, not a new story. Same argument, same words, so the film and the call agree.
- It ends on an instruction with a price on it. Start with a Longevity Day, from ■50,000, Lekki.
- Half a day at the centre. One location, three setups, one person to camera.
- Shoot Tuesday 13 or Wednesday 14 October. Final file by Monday 19 October.
- No patients. No before and after. No drug brand names. The section on what must not be in it is not negotiable.

The deadline governs everything

The film is cued in a room on a fixed date. There is no version of this where it slips.

Brief issued	Friday 9 October
Confirm you can crew it	Saturday 10 October
Recce, in person or by video	Monday 12 October
Shoot	Tuesday 13 or Wednesday 14 October
First cut	Friday 16 October
Notes back to you	Saturday 17 October
Final file delivered	Monday 19 October
Technical check at Capital Club	Tuesday 20 October
Event	Wednesday 21 October

If you cannot hold Monday 19 October, say so on Saturday. That is as useful to us as a yes.

What the film has to do

It sells. That is the job, and it is worth saying plainly because the easy version of this film is an explainer that leaves everybody better informed and nobody booked.

A guest at Capital Club or somebody who lands on the website should finish it holding four things:

- There is a gap between how long I will live and how well. It is years long and it is coming for me.
- I want what they are describing. Energy, capability, not becoming the patient my father became.
- These people measure before they prescribe. That is a reason to trust them rather than the clinic advertising a drip.
- I can start on a day, for fifty thousand naira, in Lekki. It is not a membership and it is not out of reach.

The fourth is the one most films like this leave out, and it is the one that produces a booking. The biggest barrier is not disbelief. It is a quiet assumption that longevity medicine is for people in California.

What selling does not mean here

- No promised outcome. Not a number of years, not a result, not a cure. Desire is legal; a claim is not.
- The payoff is felt, not listed. "More good years, lower risk, better quality of life" is a bullet on a slide. On film it has to be a specific picture of a life.
- It is not a clinic tour. Rooms are the setting, not the subject.
- It is not a list of services. The pyramid has six layers and naming them all in ninety seconds reads as a menu.
- No stock aspiration. No sunrises, no yoga on a beach, no stethoscope on a desk.
- It is not about the evening. It opens AGELESS and hands to Dr Ajelabi, but nothing in the picture, the words or the end card refers to the event. It has to be as right in March as it is on the 21st.

The film

Ninety seconds. Picture and sound against the clock.

The lifespan and healthspan bars on slide two of the discovery deck are the one graphic in the film. Animate them: two lines from birth, one running on, one stopping short, the gap opening between them. That gap is the proposition and it is worth eight seconds.

Time	Picture	Sound
0:00	Black. The MedLYFE mark resolves, small, centred	Room tone, then a low sustained note
0:04	Dr Ajelabi, seated, to camera, daylight	Line one
0:12	The two bars animate. Lifespan runs on. Healthspan stops short. The gap opens and holds	Music enters
0:20	Dr Ajelabi, to camera	Line two
0:32	Close: a blood sample into an analyser. Hands, gloves, no faces	Music
0:38	Close: real numbers appearing on a screen. Readouts, not motion graphics	
0:44	Dr Ajelabi, to camera	Line three
0:52	Close: a measurement being taken on a colleague, not a patient	
0:58	Dr Ajelabi, to camera	Line four
1:06	The infusion room, empty, in daylight. Slow push. Then one clean wide of the centre	Music lifts
1:12	Dr Ajelabi, to camera	Line five
1:22	Card: Start with a Longevity Day. From ■50,000. Lekki, Lagos	Music resolves
1:26	Card: MedLYFE Wellness and Longevity Centre. Live Better for Longer	
1:30	Black	Out

The last eight seconds are the ones that pay for the film. The end card is not a logo, it is an instruction with a price and a place on it.

What she says

Five lines, learned rather than read. No autocue if it can be avoided; an earpiece is better than a wandering eye. Lines three and four are lifted from the discovery deck intact.

> One. Lifespan is how long you live. Healthspan is how well you live. For most people there is a gap at the end, and it is years long.

> Two. Nobody comes to us asking to live forever. They ask to still have energy at fifty five. To be the one still travelling at seventy. To not become the patient they watched their father become.

> Three. We don't guess. We test, then we act.

> Four. We do not prescribe transformation before we understand the person.

> Five. It starts with a day. We measure you properly, you sit with a doctor, and you leave with a plan built on your own numbers rather than somebody's general advice.

If she would rather say it in her own words, that is better than these words, with two exceptions. Lines three and four are MedLYFE's own and should survive intact; they are the two sentences in the deck that sound like a position rather than a brochure.

Line two is the one to spend time on at the recce. It is the only line that makes somebody want this rather than understand it, and it will be better if it comes out of her own consulting room than out of this document. Ask her what patients actually say when they first sit down, and use that.

On camera

Who	What
Dr Adedotun Ajelabi	Head of Medicals. The only person who speaks
Two colleagues	Hands and backs of heads only, for the clinical inserts. Staff, not actors, and not patients

Nobody else is on camera. A second clinician on screen for four seconds costs an hour of the day.

Location and setups

One location, the MedLYFE centre. Three setups, which is what keeps this to half a day.

Setup	Where	For
A	A room with daylight, seated	Dr Ajelabi's three lines. The only sync sound in the film
B	The laboratory or diagnostics bench	The analyser, the sample, the screen, the measurement
C	The infusion room, and one wide architectural frame	The empty room and the exterior or atrium

If the building will not give you a wide architectural frame that flatters it, tell us at the recce and we will cut that shot rather than shoot a bad one.

Where it lives after the 21st

The event is the first outing, not the job. This is what the film is for, and it is why the versions below exist.

Where	Which version
Capital Club, 21 October	The master, projected
medlyfewellness.com, above the fold	The muted hero loop. There is no video on the site today
The first ninety seconds of every discovery call	The master, with sound
Reception, on a loop	The muted hero
Inside the sales deck	The master, embedded
Instagram, TikTok, WhatsApp	Vertical and square, captioned

A film that does six jobs is worth six times a film that does one, which is the argument for shooting it properly rather than cutting something from stills.

Deliverables

	Specification
The master	1920 x 1080, H.264, 25fps, stereo, loudness normalised. Delivered as a single named file. This is the one that gets projected on the 21st
Website hero, muted	1920 x 1080, no audio track, graded to sit behind text. Under 4MB, so it loads on Lagos 4G
Vertical	1080 x 1920, burned-in captions
Square	1080 x 1080, burned-in captions
A 30 second cut	From the same material, 16:9 and 9:16. This is the sales cut: the gap, one line of want, and the end card. For paid social and for a cold audience who will not give you ninety seconds
Stills	15 or more frames, retouched, full and web resolution
Project files and masters	Every rush, plus the project, on physical media. We will want to re-cut this in six months without re-shooting

Captions are burned in, not supplied as a sidecar. Most of this is watched with the sound off and forwarded on WhatsApp.

What must not be in it

This is the part that carries a fine, so it is written out rather than assumed.

- No patients. Nobody who could be read as a patient appears in any frame. Staff only, and only as hands and backs of heads.

- No before and after. None filmed, none supplied, none used.
- No drug or product brand names. NAFDAC does not permit prescription medicines to be advertised to the public by name. Say what something does, never what it is called.
- No claim of a result. Nothing that promises an outcome, a timescale or a cure.
- No comparison to another clinic, named or implied.
- Dr Ajelabi speaks for the centre, not for herself. The MDCN treats a practitioner advertising themselves as a disciplinary matter. She is introduced by role, and the centre is the advertiser.

One further point, for the website version only. A film shown to an invited room on one evening is not the same thing as a film running on a public website. The website version is an advertisement and sits inside ARCON's remit, which means pre-exposure approval and a licensed agency to file it. Liability is on the advertiser, not on you. Flag it to us before the hero version goes live rather than after.

What we provide

- Access to the centre for the shoot day, and for the recce on Monday 12 October.
- Dr Ajelabi, for a maximum of two hours inside the shoot day.
- Two staff for the clinical inserts.
- The MedLYFE mark, brand colours and the end card specification, including the current entry price.
- The discovery call deck, which is where the argument and two of the four lines come from. Read it before the recce.
- A named person on set who can approve a shot on the spot, so the day does not stop for a telephone call.

Please quote against these lines

Itemised, so pre-production, the day and post can be weighed separately.

1	Pre-production, recce and treatment
2	Director and crew, half day
3	Camera, lenses and support
4	Lighting and grip
5	Sound
6	Edit, to two rounds of notes
7	Grade and online
8	Sound mix and music licence
9	Motion graphics: mark resolve, the lifespan and healthspan animation, lower third, and the two end cards
10	Versioning: the master, the muted hero, vertical, square and the 30 second cut
11	Media, delivery and archive
12	Anything you believe is missing from this list

Quote the half day as a half day. If you can only crew a full day, quote that as well and we will decide.

Rights

Full, perpetual, worldwide, across every medium, to MedLYFE Wellness and Longevity Centre. Music cleared on the same basis, with the licence supplied.

You are welcome to use the finished film in your own portfolio and showreel, and we will credit you where there is a credit to give.

What we would like back on Saturday

1. Whether you can crew Tuesday 13 or Wednesday 14 October.
2. Whether you can deliver the event master by Monday 19 October.
3. The quotation.
4. Anything in this brief you would do differently. We would rather hear it now than see it in the cut.